

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Fluconazole or Clotrimazole for the treatment of Vaginal Thrush (Vulvovaginal Candidiasis)

Patient Group Direction, version 002, issued 10 September 2026. Supersedes Version 001, 1 November 2025.

Summary of NICE / NICE CKS Guidelines for Vaginal Thrush (Vulvovaginal Candidiasis)

Overview

Vulvovaginal candidiasis is a common fungal infection caused by *Candida albicans*, affecting the vulva and vaginal tissues.

Assessment

- **Symptoms:** Vulval itching, vaginal discharge (typically thick, white, curdy), vaginal soreness, vulval erythema, and pain during intercourse or micturition.

Management

- **First-line treatment:** Oral antifungal (fluconazole 150mg single dose) or topical antifungal (clotrimazole 500mg pessary single dose).

Red flags requiring referral or specialist advice

- **Recurrent infection:** 4 or more episodes per year - refer to GP for further investigation.
- **Immunocompromised patients:** Require careful management and GP referral.
- **Pregnancy:** Topical antifungal only; oral fluconazole contraindicated in pregnancy.

Patient Group Direction

for the supply/administration of

Fluconazole 150mg capsule

for the treatment of

Vaginal Thrush (Vulvovaginal Candidiasis)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
--	--

Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of uncomplicated vulvovaginal candidiasis in non-pregnant women aged 16-65 years
Inclusion criteria	Women aged 16-65 years Clinical diagnosis of uncomplicated vulvovaginal candidiasis Not pregnant or breastfeeding No contraindications
Exclusion criteria	Pregnancy Breastfeeding (relative contraindication due to insufficient data) Known hypersensitivity to fluconazole or azoles Concurrent use of terfenadine, astemizole, cisapride, pimozide, quinidine or erythromycin (risk of QT prolongation and torsades de pointes) First episode of symptoms (needs a diagnosis; refer) 4 or more episodes in 12 months, or 2 in the last 6 months (recurrent candidiasis; refer) Immunosuppression, or diabetes that is poorly controlled Abnormal or blood-stained vaginal bleeding, vulval ulcers, sores or blisters Lower abdominal pain, dysuria, fever or systemic upset, or foul-smelling discharge Possible exposure to a sexually transmitted infection, or a partner

	with an STI Aged under 16 or over 60 Severe hepatic impairment (Child-Pugh score > 9) Severe renal impairment (eGFR < 20 mL/min) History of QT prolongation or cardiac arrhythmias
Cautions	Hepatic impairment (mild to moderate) - monitor closely Renal impairment (mild to moderate) - dose adjustment may be needed Drug interactions: warfarin and other oral anticoagulants (increased anticoagulant effect), statins (increased statin levels), phenytoin (increased phenytoin levels), rifampicin (reduced fluconazole levels) Consider other medication interactions via BNF
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Fluconazole 150mg capsule
Legal category	POM
Storage	Store below 25°C in a dry place. Keep out of reach of children.
Route/method of administration	Oral, swallowed whole
Dose and frequency	Single dose of 150mg orally
Quantity to be administered and/or supplied	1 capsule (150mg)
Maximum or minimum treatment period	Single dose; no further treatment required unless symptoms persist
Adverse effects	Common: nausea, abdominal pain, diarrhoea, headache Uncommon: rash, pruritus, dizziness Rare: hepatotoxicity (raised liver enzymes, hepatitis), anaphylaxis, Stevens-Johnson syndrome, toxic epidermal necrolysis

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information:

	• that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
--	--

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Symptoms should resolve within 5-7 days. If symptoms persist or worsen, contact your GP. Wear cotton underwear and avoid irritants such as douches, scented products, and tight clothing. Avoid sexual intercourse for at least 5 days after treatment (pessary may damage condoms/diaphragms). If using the pessary, insert with fingers rather than the applicator to minimize damage to barriers. If you experience recurrent infections (4 or more per year), contact your GP for investigation. Complete the full course of treatment as prescribed. Report any adverse effects to your healthcare provider or via the Yellow Card scheme.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Vulvovaginal candidiasis

- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
10/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction
for the supply/administration of
Clotrimazole 500mg vaginal pessary
for the treatment of
Vaginal Thrush (Vulvovaginal Candidiasis)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of uncomplicated vulvovaginal candidiasis in non-pregnant women aged 16-65 years
Inclusion criteria	Women aged 16-65 years Clinical diagnosis of uncomplicated vulvovaginal candidiasis No contraindications Able to insert pessary intravaginally
Exclusion criteria	Known hypersensitivity to clotrimazole or imidazoles First episode of symptoms (needs a diagnosis; refer) 4 or more episodes in 12 months, or 2 in the last 6 months (recurrent candidiasis; refer)

	Immunosuppression, or diabetes that is poorly controlled Abnormal or blood-stained vaginal bleeding, vulval ulcers, sores or blisters Lower abdominal pain, dysuria, fever or systemic upset, or foul-smelling discharge Possible exposure to a sexually transmitted infection, or a partner with an STI Aged under 16 or over 60
Cautions	Pregnancy (topical antifungals are safe in pregnancy, but applicator should not be used; advise to insert pessary with fingers) May damage latex condoms and diaphragms - advise alternative contraception for 5 days after use Ensure patient can retain pessary (abnormal anatomy, severe prolapse)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Clotrimazole 500mg vaginal pessary
Legal category	POM
Storage	Store below 25°C in a dry place. Keep out of reach of children.
Route/method of administration	Intravaginal insertion at bedtime
Dose and frequency	Single 500mg pessary inserted intravaginally at night
Quantity to be administered and/or supplied	1 pessary (500mg)
Maximum or minimum treatment period	Single application; treatment completed in one night
Adverse effects	Common: local vaginal irritation, burning sensation, vaginal erythema Uncommon: vaginal candidiasis (rebound infection)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: that valid informed consent was given

	• name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
--	---

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Symptoms should resolve within 5-7 days. If symptoms persist or worsen, contact your GP. Wear cotton underwear and avoid irritants such as douches, scented products, and tight clothing. Avoid sexual intercourse for at least 5 days after treatment (pessary may damage condoms/diaphragms). If using the pessary, insert with fingers rather than the applicator to minimize damage to barriers. If you experience recurrent infections (4 or more per year), contact your GP for investigation. Complete the full course of treatment as prescribed. Report any adverse effects to your healthcare provider or via the Yellow Card scheme.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Vulvovaginal candidiasis
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
10/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
Name of professional group signatory	Job title & name of organisation	Signature	Date

Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
------------------	---	--	------------

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Fluconazole arm: quinidine and erythromycin added to the QT exclusion, as the SmPC contraindicates both.

Both arms: the red flags in the guidance summary and the Canesten SmPC's own 'seek medical advice' list (first episode, recurrent infection, immunosuppression, abnormal bleeding, ulcers, abdominal pain or dysuria, fever, possible STI, under 16 or over 60) were not exclusions in either arm. Added to both.

Signed on behalf of Get Real Health

Version v002, 10 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		10 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		10 September 2026

Both authorising signatories reviewed this version together on 10 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.